

**CALCAREA PHOSPHORICA IN THE TREATMENT OF  
THYROGLOSSAL DUCT CYST – A CASE REPORT****Partha Pratim Pal<sup>1\*</sup>, MD (Hom) and Souvik Dutta<sup>2</sup>, M.D. (Hom.)**

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**ABSTRACT**

Thyroglossal duct cyst is most common cyst in neck region. They occurred along the thyroid migration tract which extends from the base of the tongue through the midline of the neck to the level of the cricoid cartilage and closely associated with hyoid cartilage. In modern medicine surgical excision is performed. A 17 years aged male patient contacted over telephone with complains of firm nodular swelling on anterior part of neck. It was diagnosed as Thyroglossal cyst on ultrasonography of neck examination. To avoid surgery patient opted for homoeopathic consultation via telemedicine. The patient was treated by individualised homoeopathic medicine in LM potency on the basis of symptoms similarity and individuality. The patient had shown marked improvement of physical and mental symptoms along with disappearance of swelling on both clinically and ultrasonographic evaluation. Assessment with modified NARAnjo criteria for Homoeopathy was recorded at final consultation (+10 in ‘-6 to +13’

scale) is suggestive that it is highly probable that the patient’s improvement can be attributed to the homoeopathic treatment provided. This case report suggests that individualized homoeopathic medicine may be proved to be effective in treatment of Thyroglossal cyst without surgical intervention. More case reports, case series and randomised control trials are needed to establish the efficacy of homoeopathy in treatment of Thyroglossal cyst.

**KEYWORDS:** Thyroglossal cyst, LM potency, Individualised homoeopathic medicine, Ultrasonography.

## INTRODUCTION

The Thyroglossal cyst is the most common painless, fluctuant, mobile, nonodontogenic cyst of the neck region.<sup>[1]</sup> It is originated from epithelial remnant of Thyroglossal tract.<sup>[2]</sup> It is typically present s mobile midline neck mass near hyoid bone.<sup>[3]</sup> The Thyroglossal cyst prevails about 7% of population worldwide with an equal prevalence between male and female.<sup>[2]</sup> About 1% of populations suffering from Thyroglossal cyst have reported malignancy.<sup>[4]</sup> Although this cyst most common in pediatric population but it can be seen in people of different age groups. The swelling move ups and down while protruding tongue.<sup>[5]</sup> The cyst may be infected if not treated properly.<sup>[3]</sup> Diagnosis can be made from history and clinical examinations. Confirmatory diagnosis can be made with the help of ultrasound and computer tomography scan of neck region. The Sistrunk procedure<sup>[6]</sup> is the gold standard in this condition where removal of cyst along with part of hyoid bone is to be done.

## CASE PRESENTATION

A 17-years-old middle class male from Visakhapatnam, Andhra Pradesh consulted over telephone with complaints of firm nodular painless swelling anterior part of the neck along the midline since 6 months and was ready to avail the health services via telemedicine.<sup>[7]</sup> Along with a hoarseness of voice mainly at morning with tickling in the air passage and a foreign body sensation inside throat were also present. Apart from this he was also suffering from cramp like pain in cervical region extending up-to head since 4 months. In further enquiry, it was noted had a family history of type 2 diabetes mellitus and bronchial asthma but no such past history found. In general symptoms patient were having adequate appetite with desire for smoked meat, and having moderate thirst. Patient had a vomiting tendency and nausea while travelling by road. Bowel movements were regular. Perspiration profuse and mainly at night with offensiveness. Patient had moist cold palm. Patient having general tendency to catch cold easily with having long continued runny nose. Patient is very much restless and nervous.

## Diagnostic assessment

On high resolution ultrasound of neck, it was suggestive of Thyroglossal cyst.

### Analysis of the case and repertorization

After analysis and evaluation of the symptoms, the totality of symptoms was constructed and the case was repertorized with the help of 'Homopath Classic-Homeopathic Software version 8.0<sup>[8]</sup> premium' using Kent's repertory. The reportorial results are shown in Figure 1.

### Therapeutic Intervention with follow-ups and outcome

Based on totality of symptoms, individualization, miasmatic analysis, repertorization and materia medica consultation Silica in fifty millesimal dinamization was administered. Clinical evaluation and follow up of the patient was done quarterly or as per requirement. During follow ups repetition of doses and changes of medicine and potency (if applicable) were done as per homoeopathic guidelines. The detailed follow-up is given in Table 1.

### Outcome assessment

It was done during onset of the treatment and every follow-up. (Table1). The HOM-CASE<sup>[9]</sup> guidelines recommend that case reports treated with homeopathy should include a scientific rationale and a numerical evaluation to assess the causality of improvement. This approach is intended to provide a more rigorous and objective assessment of the effectiveness of homeopathic medical intervention. Completion of the MODified NARanjo Criteria for Homeopathy (MONARCH)<sup>[10]</sup> inventory (Table 2) was performed and the score was +10 points on the scale of -6 to +13. The completed HOM-CASE checklist is given as supplementary file 1.

## DISCUSSION

Thyroglossal cyst is basically an operable condition as there is no conventional medicinal therapy present till date.<sup>[6]</sup> In case of low risk category patient Sistrunk operation is the choice of intervention where excision of the thyroglossal duct cyst, the middle part of hyoid bone, and the surrounding tissue around the thyroglossal tract is done<sup>[11]</sup> but in high risk group total thyroidectomy with radiotherapy iodine ablation are to be considered.<sup>[12]</sup> In homoeopathy there is little evidence against a successful medicinal intervention in the case of thyroglossal cyst. A case<sup>[13]</sup> of thyroglossal cyst of a 60 years old male person was treated successfully by individualised homoeopathic medicine Thuja occidentalis in LM potency. In this case report a 17 years old male person was suffering from painless nodular firm swelling which was diagnosed as Thyroglossal cyst by ultrasonography of neck. The patient was prescribed Silicea in LM potency on the basis of symptom similarity with proper repertorization. The patient was improved symptomatically but his swelling on neck was remaining same after

administration of Silicea in series of potency from LM1 to LM 12. The ultrasonography (USG) of neck on baseline showed 2.7 x 2.5 x 1.2 cm cystic swelling located in the infrahyoid location in midline and left of midline to thyroid cartilage with internal echoes inside the cyst (Fig: 3). Moreover in another USG of neck (Fig: 4) after 1 year of treatment with Silicea LM1 to LM 12, showed not only persisting and increased thyroglossal cyst (2.6 x 2.5 x 1.5 cm) over neck region but also a colloidal cyst in right side of thyroid gland (4 x 2 mm). This colloidal cyst along with thyroglossal cyst may be due to progression of disease condition. Although the patient was symptomatically better but underlying pathology remained in same condition. In homoeopathic point of view these conditions either due to disease aggravation or some miasmatic obstacles<sup>[14,15,16]</sup> present which basically hinder the curability of this condition. If we consider the present case scenario then it showed that the patient belongs to pseudo-psoric miasmatic background with tubercular diathesis.<sup>[17]</sup> The medicine Silicea was chosen on the basis of repertorization but it not acted curatively. In Organon of Medicine,<sup>[14]</sup> Dr Samuel Hahnemann mentioned this kind of condition by stated that even after first examination of disease and formation of totality of symptoms, the first selection of medicine might not give expected result. In this case the same thing happened and Silicea though well selected but failed to act accordingly. In aphorism 169 of organon of medicine, Dr Hahnemann again quoted that if the first medicine not acted properly then after a fresh examination and thorough case taking one should prescribe indicated new medicine which may be the same medicine that was closer similimum to the medicine during 1<sup>st</sup> prescription. Here also, a new and fresh examination done and on the basis of symptom similarity Calcarea phosphorica was chosen as it was next remedy in repertorisation after Silicea. For 14 months Calcarea phosphorica was prescribed from LM1 to LM17 and due to COVID-19 patient couldn't continue medicine for 8 months. Then, in November 2020, the next 2 consecutive potencies (LM18 and 19) were sent and USG of neck was advised. In the subsequent report no abnormality was detected in the report.

**Table 1: Outcome assessments.**

| Follow up date            | Indications of prescription                                                                                                                                                                                                                                | Medicine with doses                                                                                                                                                                                  |
|---------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 20.11.2017<br>(Base line) | Firm nodular painless swelling anterior part of the neck along the midline with hoarseness of voice at morning, tickling in the air passage with a foreign body sensation inside throat. A cramp like pain in cervical region extending up-to head. Desire | Silicea 0/1 and Silicea 0/2,<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>2 phials of each potency were distributed.<br>After 0/1 finishes, 0/2 start from the next consecutive day. |

|                                           |                                                                                                                                                                                                                                                                                                                  |                                                                                                                                                                                                                              |
|-------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                           | for smoked meat. Perspiration profuse and mainly at night with offensiveness. General tendency to catch cold easily with having long continued runny nose. Patient is very much restless and nervous. Both clinically (Fig: 2) and ultrasonography (Fig: 3) suggested a case of Thyroglossal cyst.               |                                                                                                                                                                                                                              |
| 22.01.2018<br>(1 <sup>st</sup> Follow up) | Size of the swelling seems to decrease. Hoarseness of voice improved a little bit. Foreign body sensation is same as before. Cramp like pain in cervical region is unchanged.                                                                                                                                    | Silicea 0/3 and Silicea 0/4,<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>2 phials of each potency were distributed.<br>After 0/3 finishes, 0/4 start from the next consecutive day.                         |
| 26.03.2018<br>(2 <sup>nd</sup> Follow up) | Overall condition of patient is same as previously. Condition of runny nose is slightly improved with decrease in offensiveness of sweat.                                                                                                                                                                        | Silicea 0/5, Silicea 0/6, Silicea 0/7<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>2 phials of each potency were distributed.<br>After completion of one potency start new one on the next consecutive day.  |
| 25.06.2018<br>(3 <sup>rd</sup> Follow up) | Size of the swelling remains unchanged. Hoarseness of voice getting worse followed by sudden cold exposure. Foreign body sensation in throat became worse with profuse runny nose. Offensiveness of sweat became less than previous. Cramp like pain cervical region present but pain in head slightly improved. | Silicea 0/8, Silicea 0/9, Silicea 0/10<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>2 phials of each potency were distributed.<br>After completion of one potency start new one on the next consecutive day. |
| 24.09.2018<br>(4 <sup>th</sup> Follow up) | Size of swelling remains unchanged. Hoarseness of voice improved with stoppage of runny nose. Foreign body sensation in throat was disappeared with absence of tickling in throat passage. But cervical pain and runny nose present as before. Patient was advised for usg of neck.                              | Silicea 0/11 and Silicea 0/12<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>2 phials of each potency were distributed.<br>After completion of one potency start new one on the next consecutive day.          |
| 22.10.2018<br>(5 <sup>th</sup> Follow up) | On USG of neck it was seen that there was presence of small colloidal cyst in the right lobe of thyroid gland along with previous Thyroglossal cyst (Fig: 4). The size of the cyst is increased. Patient is                                                                                                      | Calcarea phos 0/1, Calcarea phos 0/2.<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>2 phials of each potency were distributed.                                                                                |

|                                            |                                                                                                                                                                                                                                                                               |                                                                                                                                                                                                                                           |
|--------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                            | nervous, restless. Desire for smoked meat. There is a profuse perspiration and coldness of palm was evident. Tendency to catch cold easily with runny nose. Hoarseness of voice also present. Cramp like pain in cervical region and nausea feeling while travelling by road. | After completion of each potency start new one on the next consecutive day.                                                                                                                                                               |
| 24.12.2018<br>(6 <sup>th</sup> Follow up)  | Swelling on neck was reduced in size. Hoarseness of voice was better than before. Cramp like pain in cervical region was improved. Nausea while travelling was persisting as usual.                                                                                           | Calcarea phos 0/3, Calcarea phos 0/4 and Calcarea phos 0/5. Marked 16 doses in aqua dist. 100ml OD for 16 days. 2 phials of each potency were distributed. After completion of each potency start new one on the next consecutive day.    |
| 25.03.2019<br>(7 <sup>th</sup> Follow up)  | Patient is improving overall.                                                                                                                                                                                                                                                 | Calcarea phos 0/6, Calcarea phos 0/7 and Calcarea phos 0/8. Marked 16 doses in aqua dist. 100ml OD for 16 days. 2 phials of each potency were distributed. After completion of each potency start new one on the next consecutive day.    |
| 24.06.2019<br>(8 <sup>th</sup> Follow up)  | Patient is improving overall. Size of swelling further decreased. Nausea was become less while travelling by road.                                                                                                                                                            | Calcarea phos 0/9, Calcarea phos 0/10 and Calcarea phos 0/11. 2 phials of each potency were distributed. Marked 16 doses in aqua dist. 100ml OD for 16 days. After completion of each potency start new one on the next consecutive day.  |
| 30.09.2019<br>(9 <sup>th</sup> Follow up)  | Patient is improving both physically and mentally.                                                                                                                                                                                                                            | Calcarea phos 0/12, Calcarea phos 0/13 and Calcarea phos 0/14. 2 phials of each potency were distributed. Marked 16 doses in aqua dist. 100ml OD for 16 days. After completion of each potency start new one on the next consecutive day. |
| 30.10.2019<br>(10 <sup>th</sup> Follow up) | Patient is improving both physically and mentally.                                                                                                                                                                                                                            | Calcarea phos 0/15, Calcarea phos 0/16 and Calcarea phos 0/17. 2 phials of each potency were                                                                                                                                              |

|                                               |                                                                                                                                                                                                                                                                                                                                                                                                                         |                                                                                                                                                                                                                                            |
|-----------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                               |                                                                                                                                                                                                                                                                                                                                                                                                                         | distributed.<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>After completion of each<br>potency start new one on the<br>next consecutive day.                                                                                |
| 11.11.2020<br>(11 <sup>th</sup> Follow<br>up) | Due to covid-19 pandemic situation,<br>it was not possible to continue<br>treatment via telemedicine and on<br>currier basis. Patient was<br>improving. There was no visible<br>swelling present on the neck region<br>(Fig: 5). As per patient's report the<br>condition of tendency to catch cold<br>easily and runny nose were very<br>less compare to previous year. The<br>patient was advised for USG of<br>neck. | Calcarea phos 0/18, Calcarea<br>phos 0/19.<br>2 phials of each potency were<br>distributed.<br>Marked 16 doses in aqua dist.<br>100ml OD for 16 days.<br>After completion of each<br>potency start new one on the<br>next consecutive day. |
| 03.02.2021<br>(12 <sup>th</sup> Follow<br>up) | USG of neck evident normal study<br>and there was no trace of colloidal<br>cyst present (Fig: 6). There was no<br>hoarseness of voice since last 6<br>months. The nausea feeling though<br>slightly persists.                                                                                                                                                                                                           | Placebo was prescribed                                                                                                                                                                                                                     |

**Table 2: MODified NARanjo Criteria for Homeopathy (Monarch).**

| Domains | Monarch                                                                                                                                                         | Answers<br>of the<br>patient | Scores |
|---------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------|--------|
| 1.      | Was there an improvement in the main symptom or condition for which the homoeopathic medicine was prescribed?                                                   | Yes                          | +2     |
| 2.      | Did the clinical improvement occur within a plausible time frame relative to the medicine intake?                                                               | Yes                          | +1     |
| 3.      | Was there a homeopathic aggravation of symptoms?                                                                                                                | Yes                          | +1     |
| 4.      | Did the effect encompass more than the main symptom or condition (i.e. were other symptoms, not related to the main presenting complaint, improved or changed)? | Yes                          | +1     |
| 5.      | Did over all well-being improve?<br>(suggest using a validated scale or mention about changes in physical, emotional and behavioural elements)                  | Yes                          | +1     |
| 6.      | (A) Direction of cure: did some symptoms improve in the opposite order of the development of symptoms of the disease?                                           | Not sure                     | 0      |
|         | (B) Direction of cure: did at least one of the following aspects apply to the order of improvement of symptoms:—From organs of more importance to               | Not sure                     | 0      |



|                                           |                                                                                                                                                                                                                                      |          |    |
|-------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------|----|
|                                           | those of less importance?<br>—From deeper to more superficial aspects of the individual?—From the top downwards?                                                                                                                     |          |    |
| 7.                                        | Did 'old symptoms' (defined as non-seasonal and non-cyclical symptoms that were previously thought to have resolved) reappear temporarily during the course of improvement?                                                          | Yes      | +1 |
| 8.                                        | Are there alternative causes(i.e. other than the medicine) that—with a high probability—could have produced the improvement?(Consider known course of disease, other forms of treatment and other clinically relevant interventions) | No       | +1 |
| 9.                                        | Was the health improvement confirmed by any objective evidence?(e.g. investigations, clinical examination)                                                                                                                           | Yes      | +2 |
| 10.                                       | Did repeat dosing, if conducted, create similar clinical improvement?                                                                                                                                                                | Not sure | 0  |
| Total score: +10                          |                                                                                                                                                                                                                                      |          |    |
| Note: Maximum score +13, minimum score -6 |                                                                                                                                                                                                                                      |          |    |

| Repertorisation: Normal                                                     |     |        |       |     |      |       |     |      |      |     |        |  |
|-----------------------------------------------------------------------------|-----|--------|-------|-----|------|-------|-----|------|------|-----|--------|--|
| Remedy Name                                                                 | Sil | Calc-p | Sulph | Lic | Phos | Nat-m | Pub | M-ac | M-hc | Sep | Carb-v |  |
| Totality                                                                    | 24  | 21     | 20    | 19  | 17   | 16    | 16  | 15   | 15   | 15  | 14     |  |
| Symptom Covered                                                             | 10  | 9      | 8     | 8   | 9    | 8     | 7   | 6    | 5    | 5   | 7      |  |
| [KT] [Mind] Restlessness, nervousness:                                      | 3   | 3      | 3     | 3   | 1    | 2     | 3   | 2    | 3    | 3   | 2      |  |
| [KT] [Stomach] Desires Meat Smoked:                                         |     | 2      |       |     |      |       |     |      |      |     |        |  |
| [KT] [Extremities] Coldness Hands:                                          | 1   | 3      | 3     | 3   | 2    | 3     | 3   | 2    | 3    | 3   | 3      |  |
| [KT] [Perspiration] Profuse Night:                                          | 3   | 2      | 3     | 2   | 3    | 2     |     | 3    | 3    |     | 2      |  |
| [KT] [Perspiration] Odour Offensive:                                        | 3   |        | 3     | 3   | 2    |       | 3   | 3    | 3    | 3   | 2      |  |
| [KT] [Generalities] Cold Tendency to take:                                  | 3   | 3      | 2     | 3   | 2    | 3     | 2   | 3    | 3    | 3   | 2      |  |
| [KT] [Larynx and Trachea] Foreign substance, sensation, larynx:             | 2   |        |       |     | 2    | 2     |     |      |      |     |        |  |
| [KT] [Larynx and Trachea] Tickling in the air passages Throat-pit, in from: | 2   |        |       |     | 1    | 1     | 2   |      |      |     |        |  |
| [KT] [Larynx and Trachea] Voice Hoarseness Morning:                         | 2   | 3      | 3     | 1   | 3    | 2     |     | 2    |      |     | 1      |  |
| [KT] [Nose] Coryza Chronic, long-continued:                                 | 2   | 1      | 2     | 2   | 1    | 1     | 1   |      |      |     |        |  |
| [KT] [Stomach] Nausea Riding in a carriage or on the cars While:            |     | 2      | 1     | 2   |      |       |     |      |      | 3   |        |  |
| [KT] [Back] Pain Cramp-like Cervical region:                                |     | 2      |       |     |      |       |     |      |      |     |        |  |
| [KT] [Back] Pain Cervical region Extending Head:                            | 3   |        |       |     |      |       | 2   |      |      |     | 2      |  |

Figure 1: Repertorization sheet.

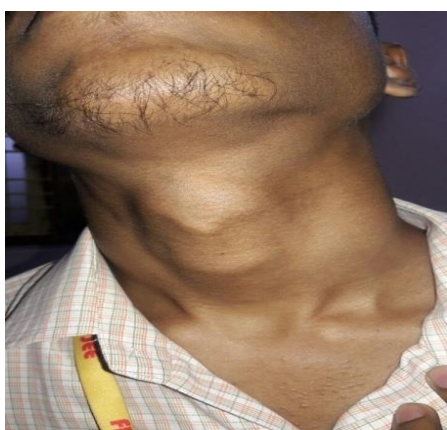


Figure 2: Front view.

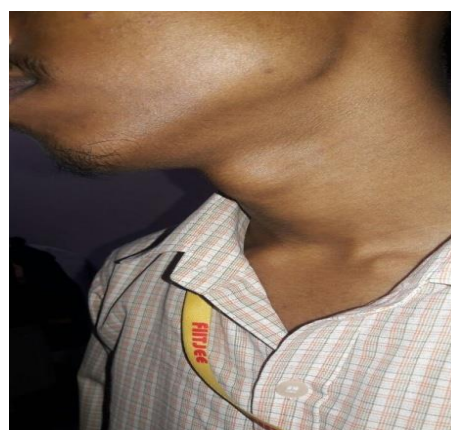


Figure 2: Lateral view.



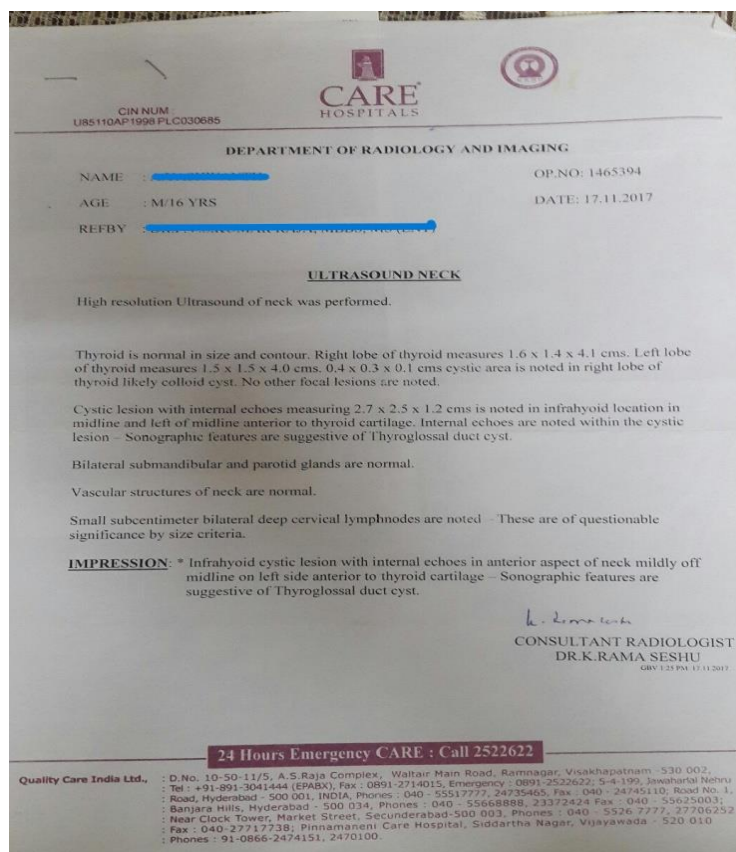
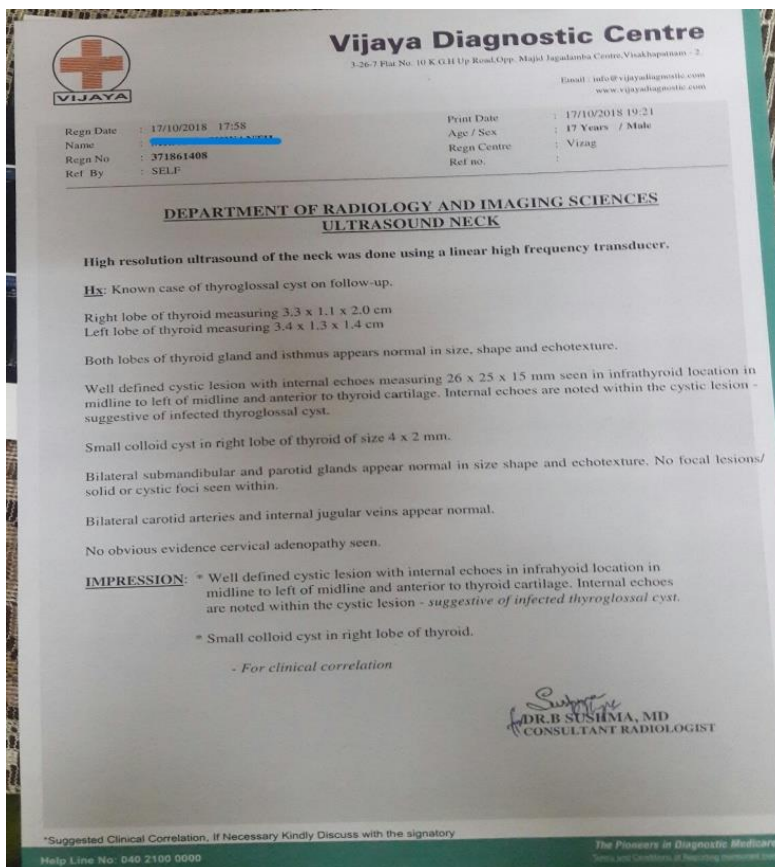
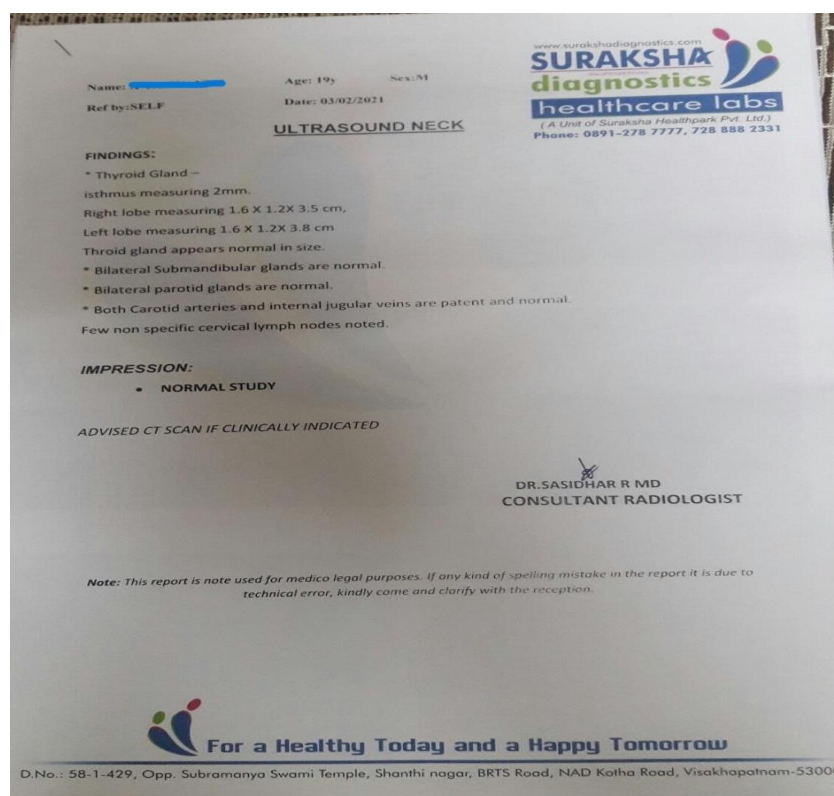
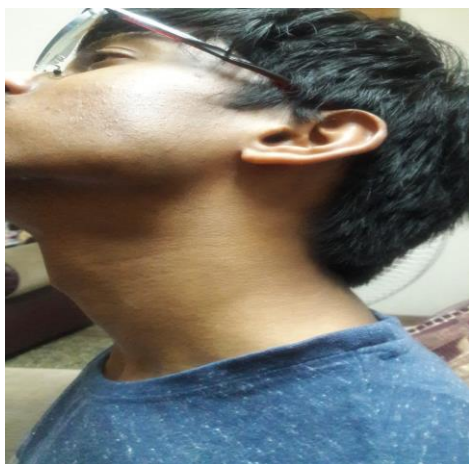


Figure 3: Ultrasonography (Base line).

Figure 4: Ultrasonography (5<sup>th</sup> Follow up).



**Figure 6: Ultrasonography (12<sup>th</sup> Follow up).**



**Figure 5: Lateral view.**



**Figure 5: Front view.**

## CONCLUSION

This case report suggests homoeopathic treatment as a promising complementary and alternative medicinal (CAM) therapy and emphasizes over the individualization and the need of repertorization in individualized homoeopathic prescription. This case shows a positive role of individualized homoeopathy in the treatment of Thyroglossal cyst. Randomized control trial is suggested with proper methodological rigor to establish efficacy of homoeopathy.

Declaration of patient consent: The authors certify that they have obtained all appropriate patient consent for using clinical information reporting in the journal. The patient understand that his name and initial will not be published and due effort will be made to conceal the identity.

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